

Clinical Study Summary Report (CSSR)

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1. Administrative & Study Identification

Full Study Title: MagnetisMM-32: A Study to Learn About the Study Medicine Called Elranatamab in People With Multiple Myeloma (MM) That Has Come Back After Taking Other Treatments (Including Prior Treatment With an Anti-CD38 Antibody and Lenalidomide)

Official Title: A PHASE 3, OPEN-LABEL STUDY OF ELRANATAMAB MONOTHERAPY VERSUS ELOTUZUMAB, POMALIDOMIDE, DEXAMETHASONE (EPd) OR POMALIDOMIDE, BORTEZOMIB, DEXAMETHASONE (PVd) OR CARFILZOMIB, DEXAMETHASONE (Kd) IN PARTICIPANTS WITH RELAPSED/REFRACTORY MULTIPLE MYELOMA WHO RECEIVED PRIOR ANTI-CD38 DIRECTED THERAPY

Short Title/Acronym: MagnetisMM-32

Protocol Number: C1071032

NCT Number: NCT06152575

Posting ID: 276240435228367103

Trial Status: RECRUITING

Sponsor Name / Company: Pfizer

Investigational Product: Elranatamab (vs. Investigator's Choice of EPd, PVd, or Kd)

Phase of Development: PHASE3

Medical Monitor: Pfizer Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective: To evaluate whether elranatamab monotherapy can provide superior clinical benefit (Progression-Free Survival and Overall Survival) compared to standard-of-care combination therapies (EPd, PVd, or Kd) in patients with relapsed/refractory multiple myeloma (RRMM).

Secondary Objectives: To evaluate progression-free survival on next line of therapy (PFS2), Objective Response Rate (ORR), Duration of Response (DOR), Very Good Partial Response (VGPR) rate, Complete Response (CR) rate, Time to Response (TTR), Minimal Residual Disease (MRD) negativity rate and duration, safety, pharmacokinetics, and health-related quality of life (HRQoL) outcomes.

2.2 Background & Rationale

To address the need for effective treatments in patients with RRMM who have previously received 1 to 4 prior lines of therapy, including an anti-CD38 directed therapy and lenalidomide. Elranatamab (ELRA), a BCMA-CD3 bispecific antibody, has shown efficacy and manageable safety as a monotherapy. This study aims to determine whether ELRA provides superior clinical benefit compared to existing regimens in early relapse.

3. Study Design & Methodology

3.1 Design Framework

Study Type: Interventional

Control Type: Active Comparator (Investigator's choice of EPd, PVd, or Kd)

Blinding: Open-label

Randomization: 1:1, Stratified by prior line of therapy (1 vs. 2 vs. 3/4) and International Staging System (ISS) disease stage (1/2 vs. 3).

3.2 Planned Enrollment & Duration

Target Enrollment: 492 participants

Number of Sites: Multicenter (Global)

Estimated Study Start: Late 2023 / Early 2024

Estimated Primary Completion: [TBD based on event-driven PFS/OS accrual]

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

1. Age ≥ 18 years with a prior diagnosis of multiple myeloma defined by International Myeloma Working Group (IMWG) criteria.
2. Measurable disease defined as at least 1 of the following: Serum M-protein ≥ 0.5 g/dL, Urinary M-protein excretion ≥ 200 mg/24 hours, or Serum involved immunoglobulin FLC ≥ 10 mg/dL with abnormal ratio.
3. Previously received 1 to 4 prior lines of therapy including a prior anti-CD38 antibody and prior lenalidomide, with documented evidence of progressive disease or failure to achieve a response to the last line of therapy.

4.2 Exclusion Criteria

1. Smoldering multiple myeloma, plasma cell leukemia, amyloidosis, or POEMS syndrome.
2. Known central nervous system (CNS) involvement or clinical signs of myelomatous meningeal involvement.
3. Stem cell transplant within 12 weeks prior to enrollment, active graft versus host disease, or previous treatment with a BCMA-directed therapy or CD3-redirecting therapy.

5. Intervention & Treatment Plan

5.1 Investigational Regimen

Dosage/Frequency: Elranatamab is given via 2 step-up priming doses, followed by weekly doses, and subsequently less frequent doses in 28-day cycles.

Route of Administration: Subcutaneous injection (Elranatamab). Comparators are given via oral, subcutaneous, or intravenous routes depending on the selected regimen (EPd, PVd, or Kd).

Duration of Treatment: Until disease progression, unacceptable toxicity, withdrawal of consent, loss to follow-up, or study termination.

5.2 Concomitant & Prohibited Therapy

Permitted: Standard supportive care therapies for multiple myeloma.

Prohibited: Live attenuated vaccines within 4 weeks of the first dose; administration of an investigational product within 30 days preceding the first dose.

6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :---
| :--- | :--- | | **Screening** | Day -28 to 0 | Informed Consent, Medical History, Laboratory Assessments, IMWG Criteria check | | **Baseline** | Day 1 | Randomization, First Dose (Step-up priming for Elranatamab) | | **Interim** | Every 28-day cycle | Efficacy Assessment (Response per IMWG), Safety Labs, PK, Administration of study drug | | **End of Study** | At progression/withdrawal | End of treatment evaluation, Followed by survival follow-up (telephone/visits) |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint

Definition: Progression-Free Survival (PFS) and Overall Survival (OS).

Measurement Method: Evaluated by Blinded Independent Central Review (BICR) per IMWG criteria and compared statistically via stratified log-rank tests.

7.2 Secondary & Exploratory Endpoints

- Objective Response Rate (ORR), Duration of Response (DOR), Very Good Partial Response (VGPR) rate, Complete Response (CR) rate, Time to Response (TTR).
- Minimal Residual Disease (MRD) negativity rate and duration, Safety profile, Immunogenicity, and HRQoL.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via regular clinic visits and laboratory evaluations during the 28-day cycles.

Serious Adverse Events (SAEs): Routine reporting within 24 hours of site awareness.

Data Monitoring Committee (DMC): Independent Data Monitoring Committee established.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy outcomes (PFS/OS). Safety Set for all AE evaluations.

Sample Size Justification: \$N = 492\$ appropriately powered to detect statistically significant differences in PFS and OS via

stratified log-rank tests.

Handling of Missing Data: Standard oncology conventions, with censoring applied for time-to-event efficacy analyses.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular onsite and remote monitoring visits to ensure protocol adherence and safety.

Audit Trail: Electronic Data Capture (eCRF) maintaining complete audit trails and data clarification processes.

11. Study Identifiers & Governance

Primary ID: C1071032

Registry Number: NCT06152575

Posting ID: 276240435228367103

Sponsor/Lead Organization: Pfizer

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12. Clinical Framework

Disease Areas: Multiple Myeloma

Preferred UMLS Name: Multiple Myeloma

Primary Condition: Relapsed/Refractory Multiple Myeloma (RRMM)

Diagnosis Threshold: Prior diagnosis per IMWG criteria with measurable disease (e.g., Serum M-protein ≥ 0.5 g/dL)

Medical Methodology: Bispecific Antibody Therapy (BCMA-CD3) vs. Standard of Care Combination (EPd, PVd, Kd)

Optimization Target: Progression-Free Survival (PFS) and Overall Survival (OS) prolongation

13. Study Procedures & Methodology

Management Pathway: Continuous treatment in 28-day cycles until disease progression or unacceptable toxicity

Education Protocol (HCP): Administration of step-up dosing protocol for bispecific antibodies and management of resulting toxicities

Education Protocol (Patient): Clinic visits for subcutaneous injection schedules and side effect awareness

Quality Audit Mechanism: Blinded Independent Central Review (BICR) for unbiased efficacy endpoint assessment

14. Observation & Follow-Up Schedule

Total Duration: Continuous until disease progression, followed by extended survival follow-up

Onsite Visit Cadence: Weekly during initial cycles, followed by less frequent assessments aligning with 28-day cycles

Remote Monitoring Frequency: Telephone contacts or visits after discontinuation of study treatment to monitor OS and PFS2

Checklist Review Interval: Every 28-day cycle

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---	
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]					